# 6.11 Intestinal Obstruction

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• "My abdomen is swollen and I cannot pass wind."<br>• Colicky pain, 7/10<br>**Objective**<br>• Abdomen distended<br>• Vomiting<br>• Bowel sounds high-pitched | **ACTUAL**<br>**Acute pain** related to bowel distension as evidenced by colicky pain and distension. | Within 24 hours, less pain and less distension. | • Nothing by mouth.<br>• Pass an NG tube to decompress the stomach as ordered, and record what drains.<br>• Give analgesia as prescribed.<br>• Measure abdominal girth every 4–8 hours. | Less pain, girth smaller: **goal met**. |
| **Subjective**<br>• "I keep vomiting."<br>**Objective**<br>• Mouth dry<br>• Urine output low<br>• Potassium low | **ACTUAL**<br>**Deficient fluid volume** related to vomiting and fluid sequestration in the bowel as evidenced by dry mucosa and low urine output. | Within 24 hours, urine output adequate and electrolytes normal. | • Give IV fluids and potassium as prescribed.<br>• Keep an I&O chart, counting NG losses.<br>• Follow electrolytes. | Balanced: **goal met**. |
| **Subjective**<br>• *No specific complaint*<br>**Objective**<br>• Bowel completely obstructed<br>• HR climbing | **RISK**<br>**Risk for shock** as evidenced by risk factors: strangulation, bowel ischemia and fluid loss. | Signs of strangulation are picked up early. | • Report constant pain, fever, a rising HR or signs of peritonism.<br>• Get him ready for surgery as ordered. | No shock: **goal met**. |